RMNCH + A Program

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RMNCH + A

Reproductive, Maternal, Newborn, Child & Adolescent Health (RMNCH+A) Programme



Introduction

- Mothers and children not only constitute a large group, but they are also **“vulnerable” or special group.**
- They comprises **71.4 %** of population of the developing countries.
- In India, women of child bearing age (15-44 years) constitute **22.2%** and children under 15 years of age about **35.3%** of total population, together **57.5%** of population consists of mothers and children.

Definition

According to **WHO (1976),**

- Maternal and child health services can be defined as
- “promoting, preventing, therapeutic or rehabilitation facility or care for the mother and child”

Historical Back Ground

- **1950** : MCH services as a basic health service in PHC
- **1952**: National Family Planning Programme
- **1961** : Department of family planning created in the ministry of health.
- **1966**: All India Hospital Post Partum Programme
- **1971 :** Medical termination of pregnancy (MTP) act.

Historical Back Ground

- **1977**: National Family Welfare Programme
  (Renaming of family planning to family welfare.)
- **1978 :** Expanded programme on immunization.
- **1985 :** Universal immunization programme + national oral rehydration therapy programme.
- **1992**: National Child Survival & Safe Motherhood (CSSM) program
- **1996** : Target free approach.

Historical Back Ground

- **1997**: RCH phase – I
- **2005**: NRHM
- **2OO5** : RCH phase - II
- **2013** : NHM
- **2013** : RMNCH + A
- **2014 :** India Newborn Action Plan (INAP)

Recent trends in MCH care

- Integration of care
- Risk approach
- Manpower changes
- Primary health care
- Reproductive and child health



- June 2012 : "Global Child Survival Call to Action : A Promise to Keep" at Washington DC
- By UNICEF, Govt. of India, Ethiopia & USA,
- More than 80 countries
- Target : Child mortality to ≤20 child deaths per 1000 live births in every country, by 2035…

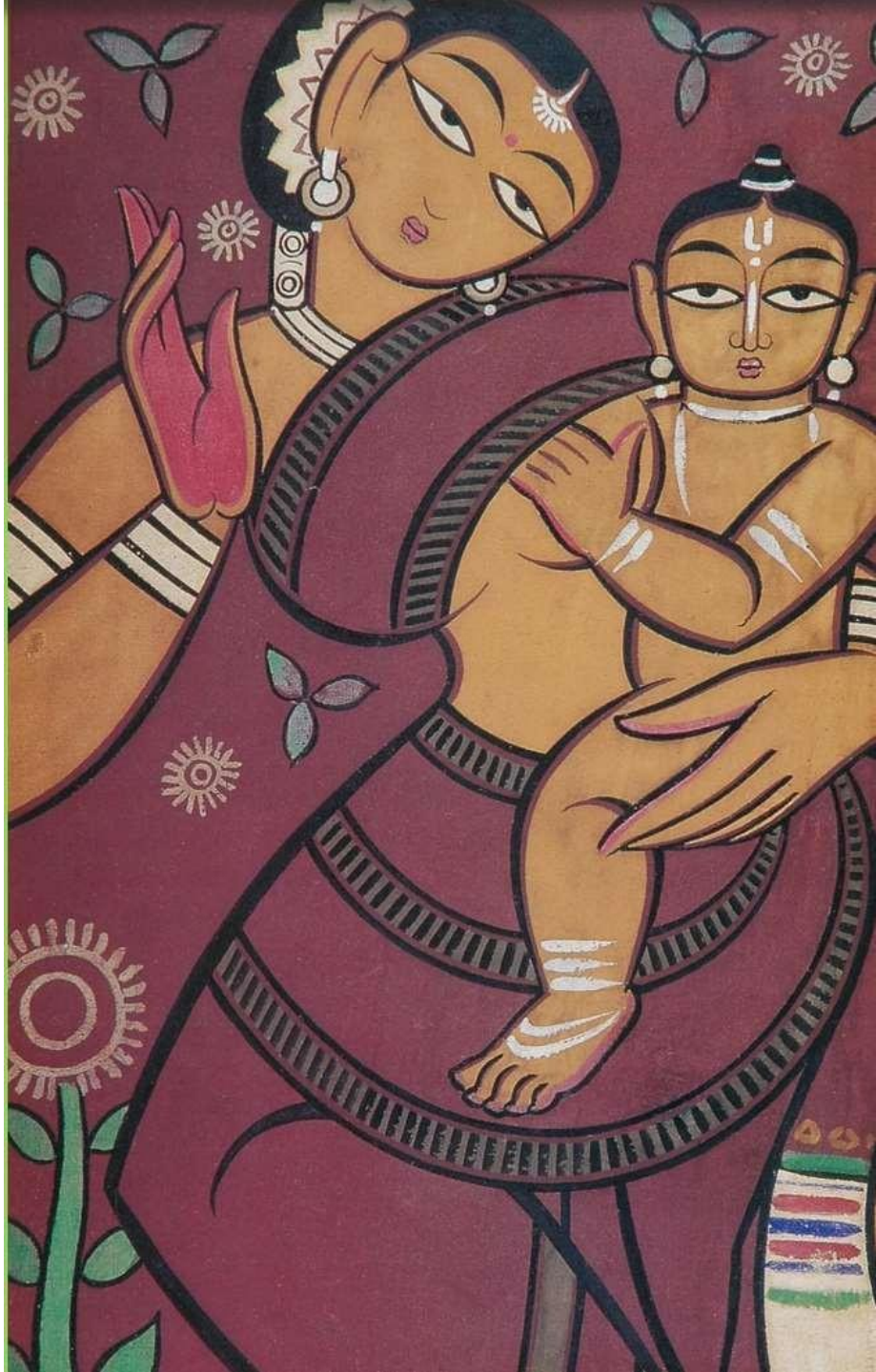
8 months Later…

In February 2013 :

The Launch of :

A Strategic Approach to Reproductive, Maternal , Newborn , Child, and Adolescent Health (RMNCH +A)

Heart ♥ of National Health Mission



A Strategic Approach to Reporoductive, Maternal, Newborn, Child and Adolescent Health (RMNCH+A) in India



For Healthy Mother and Child

Ministry of Health & Family Welfare
Government of India
January, 2013

Also Aided by…

- USAID and its Maternal Child Health Integrated Programme (MCHIP)



- UNICEF





- UNFPA



- NIPI



RMNCH+A

What is RMNCH+A Strategic approach?

1. This is a comprehensive strategy for improving the maternal and child health outcomes , under NRHM

2. It is based on the evidence that maternal and child health cannot be improved in isolation as adolescent health and family planning have an important bearing on the outcomes.

3. This strategy encompasses various high impact interventions across the life cycle.

4. The strategy is based on the concept of '**CONTINUUM OF CARE'**

Why '+' ???

- Adolescence – *First time* as a Distinct life stage

- *Linking* Maternal and Child health to Reproductive Health, Family Planning, Adolescent health,HIV, Gender Preconception, Prenatal Diagnostic Techniques

- *Linking* Home and Community based services to Facility based care…

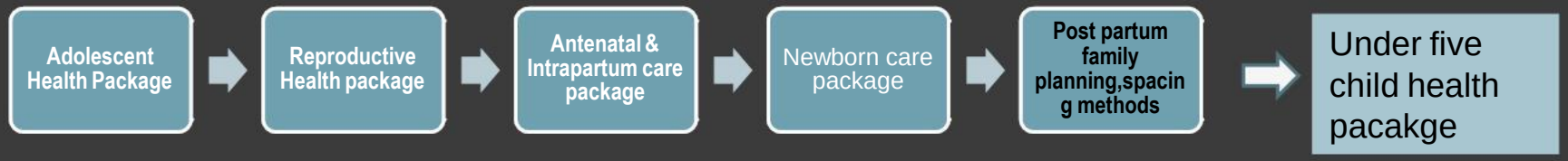
- Ensuring Linkages, Referrals and Counter-Referrals between Primary, Secondary and Tertiary levels…

What does RMNCH+A stands for

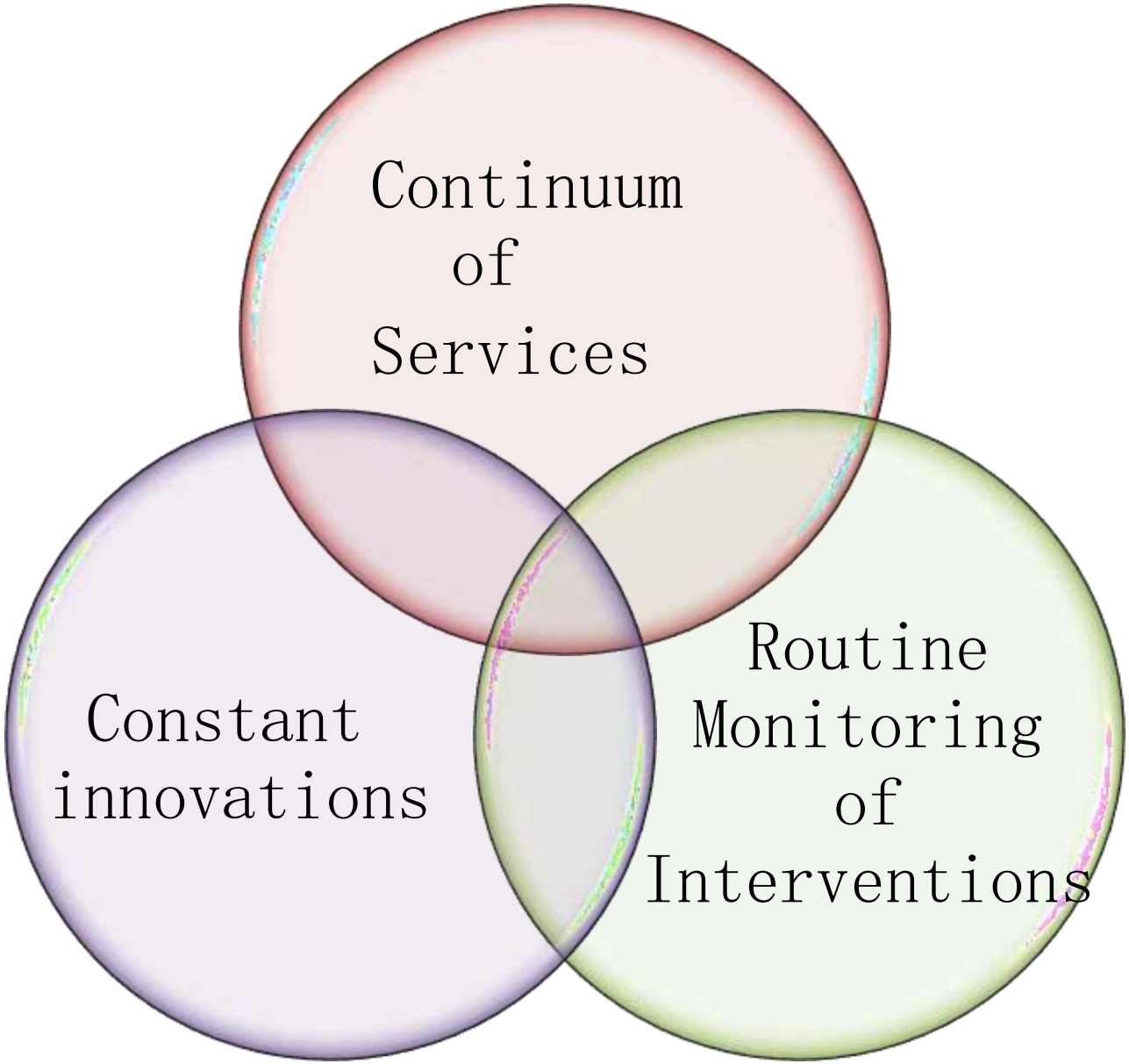


- **Reproductive, Maternal, New-born, Child & Adolescent Health** : Links maternal and child survival to other components (family planning , adolescent health, gender & PC & PNDT)

- **Plus denotes**
- inclusion of **adolescence** as a distinct ‘life stage’ in the overall strategy
- **Links community and facility based care as well as referrals between various levels of health care system**



Reach the maximum number of people, in the most remote corners of the Country through :-



HIGH PRIORITY DISTRICT

- Relative ranking of districts has been done within a State (based on a composite index) and bottom 25% of the districts be selected as High Priority Districts for that State.

- 184 High Priority Districts (HPDs) were selected in 29 states based on **COMPOSITE HEALTH INDEX.**

Strategic approach: Reaching geographies & populations with highest burden of mortality

Identification of 184 High Priority Districts (HPDs)

Based on a 'composite health index' across States

Differential planning for HPDs

30% higher financial allocation; differential norms for HR, infrastructure; incentives for HR

Harmonisation of technical assistance

All partners to work with States to accelerate progress on RMNCH+A

Management tools and Job aids :

- **5×5 matrix**
  - 5 High impact interventions in 5 thematic areas
  - 5 cross cutting interventions
  - 5 health system strengthening interventions

5 X 5 matrix for High Impact RMNCH+A Interventions When Implemented with High Coverage and High Quality

Reproductive Health	Maternal Health	Newborn Health	Child Health	Adolescent Health
Health Systems Strengthening			Cross cutting Interventions	

5 X 5 matrix for RMNCH+A

Let's make every mother and child healthy,

Transformational Leadership can do it

Reproductive Health

- Focus on spacing methods, particularly PPIUCD at high case load facilities
- Interval IUCD at sub-centers on fixed days
- Doorstep delivery of contraceptives by ASHA
- Strengthening safe abortion services
- Maintaining sterilization services

Maternal Health

- Use MCTS to ensure early registration of pregnancy and provide full ANC
- Detect high risk pregnancies and line list and manage severely anemic mothers
- Equip delivery points with trained HR & other infrastructure
- Review maternal, infant and child deaths for corrective actions
- Notify sub-centers with less institutional delivery load, distribute Mesoprostol and incentivize ANMs for domiciliary deliveries

Newborn Health

- Early initiation and exclusive breastfeeding
- Home based newborn care through ASHA
- Essential Newborn Care and resuscitation services at all delivery points
- Equip Special Newborn Care Units with highly trained HR and other infrastructure
- Empower ANM for community level use of Gentamycin

Child Health

- Complementary feeding, IFA supplementation and focus on nutrition
- Diarrhoea management at community level using ORS and Zinc
- Management of pneumonia
- Full immunization coverage
- Rashtriya Bal Swasthya Karyakram (RBSK): screening of children for 4D (birth defects, development delays, deficiencies and disease) and its management

Adolescent Health

- Community based services through peer educators
- Delay in age of marriage
- Strengthen ARSH clinics
- Weekly IFA Supplementation (WIFS) under national Iron Plus initiative
- Promote menstrual hygiene

Health Systems

- Case load based deployment of HR at all levels
- Ambulances, drugs, diagnostics, reproductive health commodities
- Behavior change communication
- Supportive supervision and use of scorecards based on HMIS
- Public grievances redressal mechanism

Cross cutting

- Equip nurses to provide specialized and quality care
- Address social determinants of health through convergence
- Introduce difficult area and performance based incentives
- Focus on un-served and underserved villages, urban slums and blocks
- Bring down out of pocket expenses

5 X 5 Matrix for High Impact RMNCH+A Interventions

To be Implemented with High Coverage and High Quality

Reproductive Health	Maternal Health	Newborn Health	Child Health	Adolescent Health
				

RMNCH+A approach… key features

Heath Systems strengthening : Infrastructure, Human resources, drugs & commodities, referral transport

Five high impact interventions across five key life stages

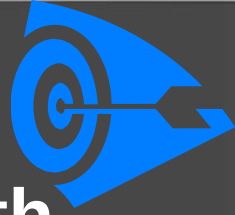
Convergence & partnerships

with other Ministries/ departments, development partners, civil society organisations & other stakeholders

Prioritisation of investments : High Priority Districts, tribal blocks, marginalised populations in underserved areas …also urban slums

Integrated monitoring and accountability through good governance, use of data, communitisation & grievance redressal

GOALS



Health outcome goals established in the 12th Five Year Plan.

- Reduction of Infant Mortality Rate (IMR) to 25 per 1,000 live births by 2017
- Reduction in Maternal Mortality Ratio (MMR) to 100 per 100,000 live births by 2017
- Reduction in Total Fertility Rate(TFR) to 2.1 by 2017

Coverage targets for key RMNCH+A interventions for 2017

Coverage targets for key RMNCH+A interventions for 2017

- Increase facilities equipped for perinatal care (designated as 'delivery points') by 100%
- Increase proportion of all births in government and accredited private institutions at annual rate of 5.6 % from the baseline of 61% (SRS 2010)
- Increase proportion of pregnant women receiving antenatal care at annual rate of 6% from the baseline of 53% (CES 2009)
- Increase proportion of mothers and newborns receiving postnatal care at annual rate of 7.5% from the baseline of 45% (CES 2009)
- Increase proportion of deliveries conducted by skilled birth attendants at annual rate of 2% from the baseline of 76% (CES 2009)

- Increase exclusive breast feeding rates at annual rate of 9.6% from the baseline of 36% (CES 2009)
- Reduce prevalence of under-five children who are underweight at annual rate of 5.5% from the baseline of 45% (NFHS 3)
- Increase coverage of three doses of combined diphtheria-tetanus-pertussis (DTP3) (12–23 months) at annual rate of 3.5% from the baseline of 7% (CES 2009)
- Increase ORS use in under-five children with diarrhoea at annual rate of 7.2% from the baseline of 43% (CES 2009)

- Reduce unmet need for family planning methods among eligible couples, married and unmarried, at annual rate of 8.8% from the baseline of 21% (DLHS 3)
- Increase met need for modern family planning methods among eligible couples at annual rate of 4.5% from the baseline of 47% (DLHS 3)
- Reduce anaemia in adolescent girls and boys (15–19 years) at annual rate of 6% from the baseline of 56% and 30%, respectively(NFHS 3)
- Decrease the proportion of total fertility contributed by adolescents (15–19 years) at annual rate of 3.8% per year from the baseline of 16% (NFHS 3)
- Raise child sex ratio in the 0–6 years age group at annual rate of 0.6% per year from the baseline of 914 (Census 2011)

Rational of RMNCH A+ strategies

- In order to bring greater impact through RCH programme, it is important to recognise that reproductive, maternal and child health cannot be addressed in isolation

- RMNCH+A strategic approach focuses on what the Health Delivery System can do to help achieve maternal and child health goals

- Purpose RMNCHA +A approach is to provide an understanding of comprehensive approach to improve child survival and safe motherhood

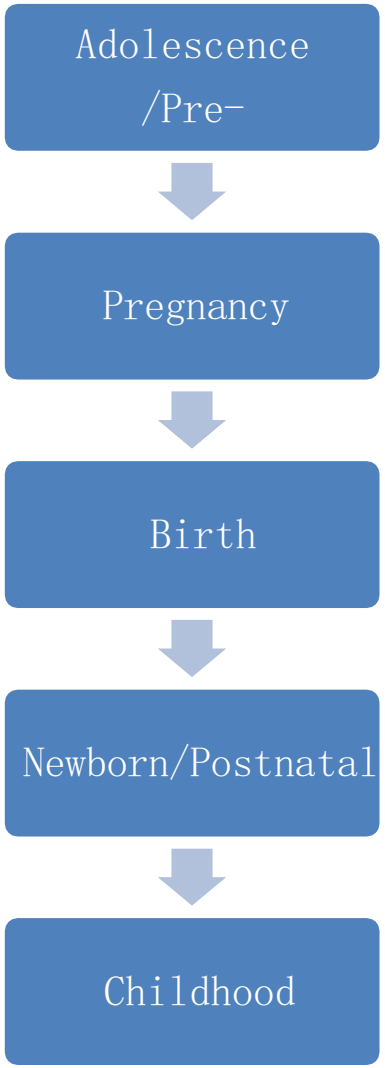
The Implementation Strategies of RMNCH+A

Implementation

Continuum of care…

- Across the Life cycle
- Across Different levels of Health System…

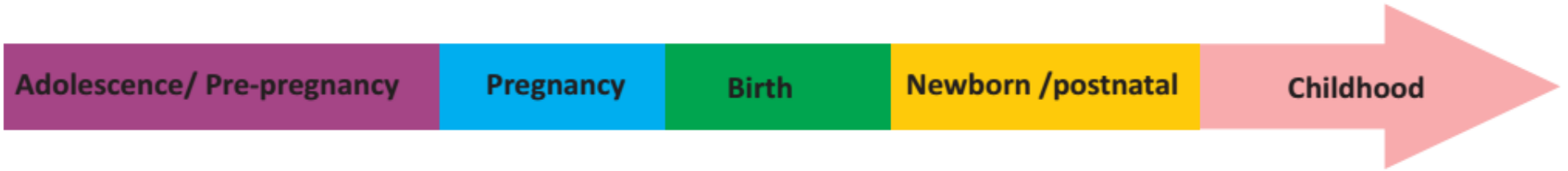




Continuum of Care

	Reproductive care	Pregnancy and child birth care	Newborn and childcare
Clinical	• Comprehensive abortion care • RTI/STI case management, • Postpartum IUCD and sterilisation; interval IUCD procedures • Adolescent friendly health services	• Skilled obstetric care and immediate newborn care and resuscitation • Emergency obstetric care • Preventing Parent to Child Transmission (PPTCT)of HIV • Postpartum sterilisation	• Essential newborn care • Care of sick newborn (SNCU, NBSU) • Facility-based care of childhood illnesses (IMNCI) • Care of children with severe acute malnutrition (NRC) • Immunisation

	Reproductive health care	Antenatal care	Postnatal care	Child health care
Outreach/Sub centre	• Family planning (including IUCD insertion, OCP and condoms) • Prevention and management of STIs • Peri-conception Folic acid supplementation	• Full antenatal care package • PPTCT	• Early detection and management of illnesses in mother and newborn • Immunisation	• First level assessment and care for newborn and childhood illnesses • Immunisaton • Micro-nutrient supplementation
Family & Community	• Weekly IFA supplementation • Information and counselling on sexual reproductive health and family planning • Community based promotion and delivery of contraceptives • Menstrual hygiene	• Counselling and preparation for newborn care,breast feeding, birth preparedness • Demand generation for pregnancy care and institutional delivery (JSY, JSSK)	• Home-based newborn care and prompt referral (HBNC scheme) • Antibiotic for suspected case of newborn sepsis • Infant and Young Child Feeding (IYCF), including exclusive breast feeding and complementary feeding, • Child health screening and early intervention services (0–18 years) • Early childhood development • Danger sign recognition and care-seeking for illness • Use of ORS and Zinc in case of diarrhoea	
	Intersectoral: Water, sanitation, hygiene, nutrition, education, empowerment			



Adolescent Health Programme

ADOLESCENT

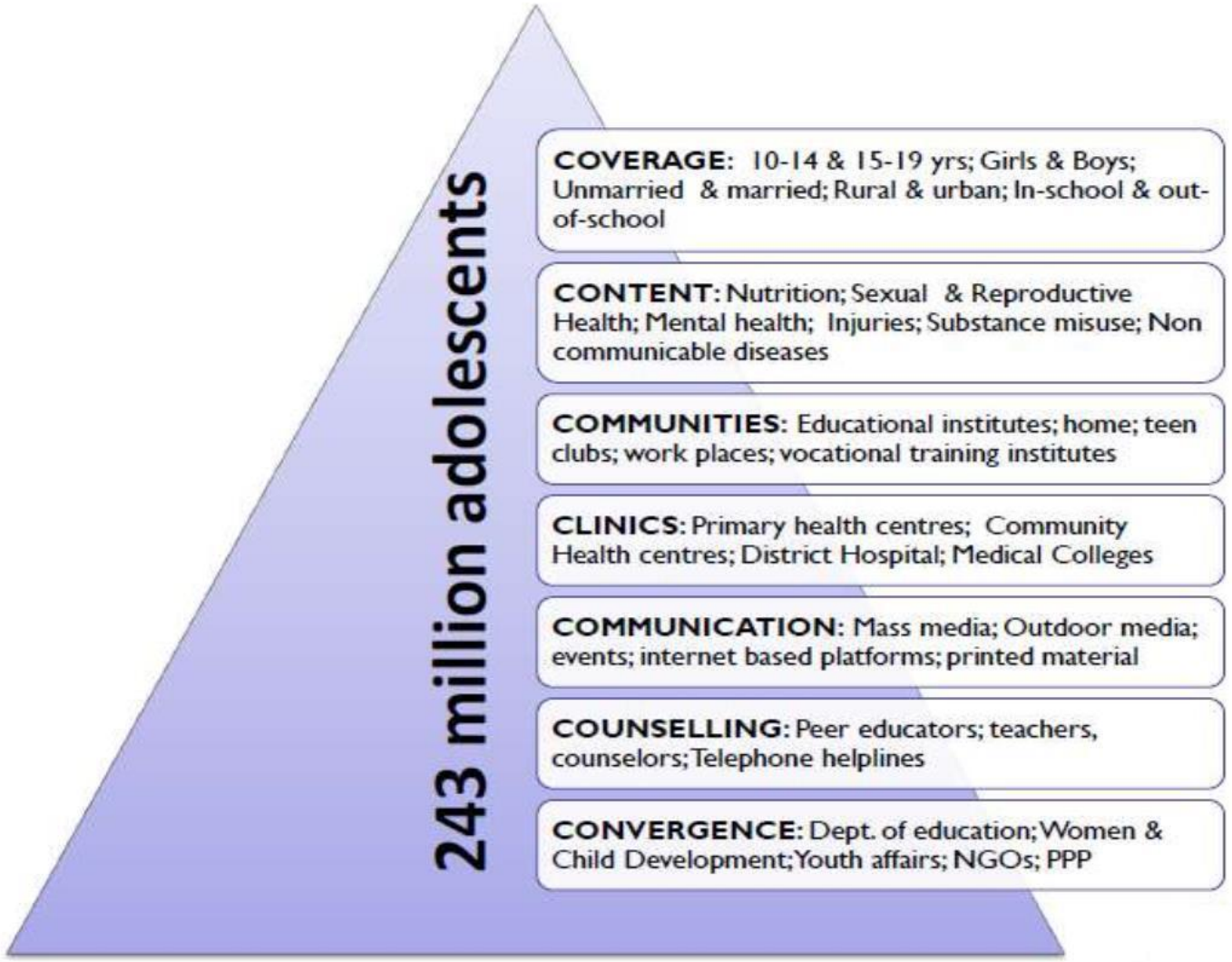
Priority interventions:

1. Adolescent nutrition; iron and folic acid supplementation
2. Facility-based adolescent reproductive and sexual health services (Adolescent health clinics)
3. Information and counseling on adolescent sexual reproductive health and other health issues



4. Menstrual hygiene
5. Preventive health checkups

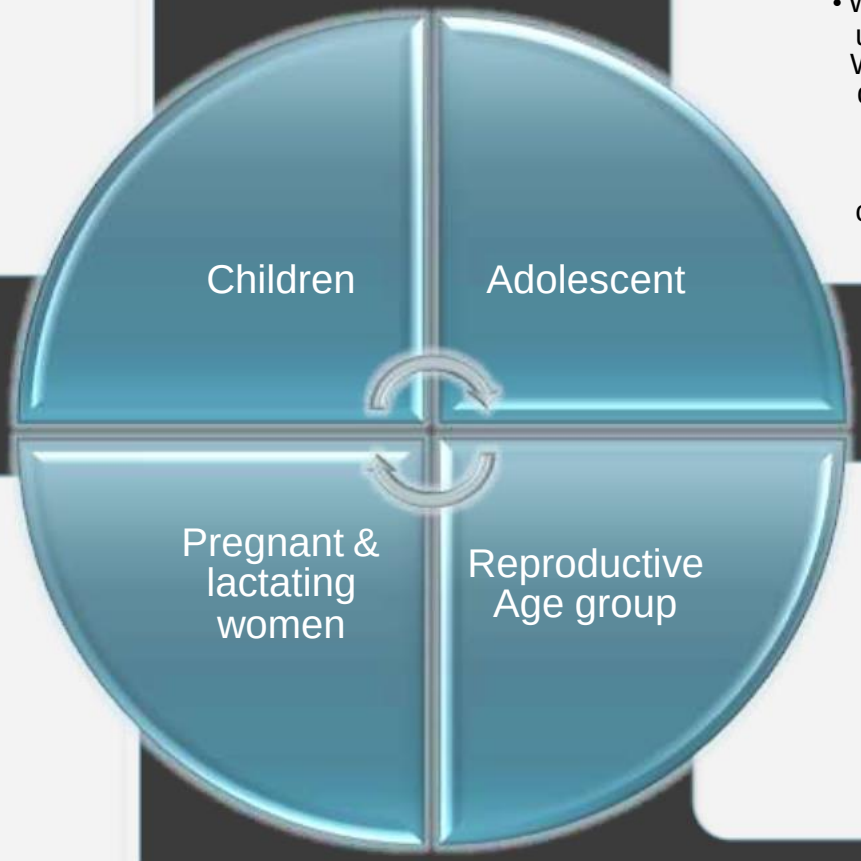
+A: National Adolescent Health Strategy - 7 Cs



1. New Initiative: National Iron Plus Initiative (NIPI)

- **6-60 months**: IFA administered biweekly, on fixed days , under direct supervision of ASHAs ;
  5-10 years: at AWC & through schools

- **NEW COMPONENT**
- Weekly IFA upplementation WIFS) for both dolescents boys & rls in overnment/Govern ent aided/municipal chools



- As part of the antenatal care package, at all levels of health facilities , sub centre and outreach

- **NEW COMPONENT**
- IFA to be distributed by ASHA during doorstep delivery of contraceptives; IFA tablets to be given for 52 weeks each year

2. Adolescent Friendly Health Services (Adolescent Health Clinics)

- Services at **sub centre: ANM**
- Adolescent Information and Counseling Centre will be made functional by MO and ANM at PHC on **weekly basis.**
- At CHC, DH/SDH/ and Medical College: Adolescent Health Clinics(daily basis)
- Special focus will be given to establishing linkages with Integrated Counseling and Testing Centres (ICTCs) and making appropriate referrals for HIV testing and RTI/STI management

3. Information & Counselling

- On sexual reproductive health
- On other health issues

4. Scheme for promotion of menstrual hygiene among adolescent girls in rural India:



- This scheme promotes better health and hygiene among adolescent girls
- Sanitary napkins are provided under NRHM"s **brand 'Free days'**.
- These napkins are being sold to adolescent girls by ASHAs

5. Preventive health checkups and screening for diseases, deficiency and disability

- Components of School Health Programme include screening, basic health services and referral
- Bi-annual health screening is undertaken for students **(6–18 years age group)**
- Implementation of School Health Programme
- Team consist of:

  - 2 Medical Officers
    (MBBS / Dental / AYUSH qualified)
  - 2 paramedics
    (one ANM and any one of the following Pharmacist/ Ophthalmic Assistant/Dental assistant)

Care During Pregnancy & Child Birth

Pregnancy and Childbirth

Priority interventions:

1. Preventive use of folic acid in peri-conception period
2. Delivery of antenatal care package and tracking of high- risk pregnancies
3. Skilled obstetric care
4. Immediate essential newborn care and resuscitation
5. Emergency obstetric and new born care
6. Postpartum care for mother and newborn
7. Postpartum IUCD and sterilisation
8. Implementation of PC&PNDT Act



1. Preventive use of folic acid in peri-conception period

- To promote use of folic acid in planned pregnancies during peri-conception phase by frontline workers and facility-based service providers.
- **A new scheme for delaying first birth after marriage and ensuring spacing between first and second child was launched in May 2012 and has provision for incentivizing ASHAs for their efforts**

2. Delivery of antenatal care package and tracking of high-risk pregnancies

- Pregnancy Testing Kits **(Nishchay)** to all sub centres and through ASHAs.
- **Universal access to full antenatal package**
- **Mother and Child Tracking system (MCTS)**
- Tracking pregnant women with severe anaemia by ANM and PHC in charge
- **Universal confidential HIV screening**

3. Skilled obstetric care and essential newborn care and resuscitation

- **Delivery points** are to be prioritised.
- Delivery points should be saturated with **Skilled Birth Attendance**
- **Janani Suraksha Yojana (JSY)**
- **Janani Shishu Suraksha Karyakram (JSSK)** is an initiative to reduce out-of-pocket expenses related to maternal and newborn care.
- **Newborn Care Corners** are established at delivery points and providers are trained in basic newborn care and resuscitation through **Navjaat Shishu Suraksha Karyakram (NSSK).**

4. Emergency obstetric & new born care

- Sub centres and PHC designated as delivery points
- CHC (FRUs) and District Hospitals have been made functional 24 X7
- **MCH Wing**, with integrated facilities for advanced obstetric and neonatal care & also ensure **48hrs stay** for mother and newborn at hospital

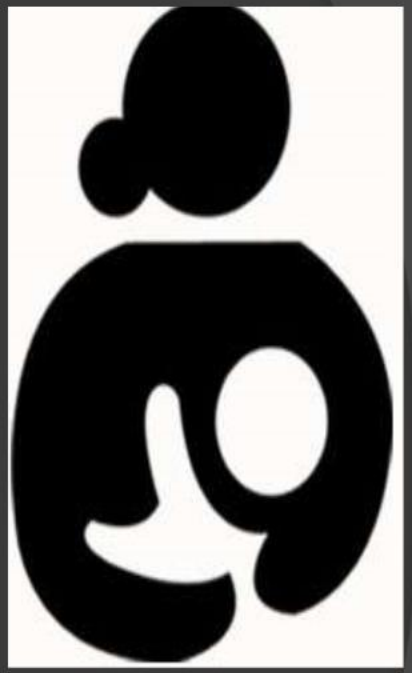
Emergency obstetric & new born care

- Multi skilling of doctors in public health system:

- **8week training** programme of MBBS qualified doctors in **Life Saving Anaesthetic Skills** (LSAS)
  - **16week** training programme **in Obstetric Management Skills including Caesarean section**
  - **10 day** training for Medical Officers in **Basic Emergency Obstetric Care (BEmOC)**
  - **3 week** Skilled Birth Attendance training for ANMs/Staff Nurses.

5. Postpartum care for mother and baby

- To ensure postpartum care for mothers and newborns, **forty-eight hours** of stay at health facility for institutional delivery
- At least **three postnatal visits to mother and six postnatal visits to newborn** are to be made within six weeks of delivery/birth



6. Postpartum IUCD insertion and sterilization



- Placement of trained providers for post-partum IUCD (PPIUCD) insertion at district and sub-district hospital level

- Training of Medical Officers in „Minilap“ for provision of Post-Partum Sterilisation in high case load facilities

- RMNCH counsellor ensure healthy timing and spacing between pregnancies

7. Implementation of preconception and prenatal diagnostic techniques (PC&PNDT) Act

- Key action:

- **F**ormation of PC&PNDT cells at state/district level,

- **S**trengthening of human resources as well as trainings & establishing appropriate infrastructure at all levels

- **B**uilding community opinion against sex selective abortion and foeticide by sensitising and mobilising self-help groups and empowering women

New Born & Child Care

Newborn and Child care

Priority interventions:

1. Home-based newborn care and prompt referral
2. Facility-based care of the sick newborn
3. Integrated management of common childhood illnesses (diarrhoea, pneumonia and malaria)
4. Child nutrition and essential micronutrients supplementation
5. Immunisation
6. Early detection and management of defects at birth, deficiencies, diseases and disability in children (0–18 years) (RBSK)

1. Home-based newborn care and prompt referral

- Home-based newborn care scheme launched in 2011 provides immediate postnatal care and essential newborn care to all newborns up to the age of 42 days
- ASHAs are trained and incentivised to provide special care to pre terms and newborns
- ASHA are also trained in identification of illnesses, appropriate care and referral through home visits

2. Facility-based care of the sick newborns

- **Special Newborn Care Units (SNCU):** District Hospitals and tertiary care hospitals
- **Goal**: one SNCU in each district of the country and in health facilities with more than 3,000 deliveries per year
- **Newborn Stabilisation Unit (NBSU)**: Community Health Centres / First Referral Units (4 beded)

3. Child nutrition and essential micronutrients supplementation



- Follow up of LBW baby by ASHA and ANM
- Bi-weekly iron and folic acid supplementation for preschool children of 6 months to 5 years as part of the National Iron + initiative.
- Administration of deworming tablets/syrup combined with Vitamin A supplementation during biannual rounds.

4. Integrated management of common childhood illnesses (pneumonia, diarrhoea and malaria

- Availability of ORS and Zinc should be ensured at all sub-centres and with all frontline workers
- Timely and prompt referral of children with fast breathing and/or lower chest in-drawing should be made to higher level of facilities.
- Training of health service providers (doctors and nurses), especially those at FRUs and District Hospitals in F-IMNCI

5. Immunization

- Second dose of measles has been introduced and Hepatitis B vaccine is now available in the entire country
- To strengthen routine immunization, newer initiatives include
  - provision for Auto Disabled (AD) Syringes to ensure injection safety
  - support for alternate vaccine delivery from PHC to sub-centres as well as outreach sessions
  - mobilization of children to immunization session sites by ASHA
- Coverage of vaccine beyond first year of life must be emphasised and monitored
- Investigation report of every serious „adverse event following immunisation" (AEFI) case must be submitted within 15 days of occurrence to district AEFI Committees

6. Child Health Screening and Early Intervention Services (Rashtriya Bal Swasthya Karyakram)

- This initiative aims to reach 27 crore children annually in the age group 0-18 years
- Child health screening and early interventions services will be provided by mobile health teams at block level
- These teams will include
  - at least 2 doctors (MBBS /AYUSH qualified)
  - 2 paramedics
- The health screening will be conducted to detect 4Ds: defects, deficiencies, diseases, development delays including disabilities

Care Through The Reproductive Years

Care Through The Reproductive Years

Priority interventions:

1. Community-based promotion and delivery of contraceptives
2. Promotion of spacing methods (interval IUCD)
3. Sterilisation services (vasectomies and tubectomies)
4. Comprehensive abortion care (includes MTP Act)
5. Prevention and management of sexually transmitted and reproductive infections (STI/RTI)



1. Community based doorstep distribution of contraceptives

- ASHAs are utilised to deliver contraceptives at the doorstep of households.
- ASHA charges a nominal amount from beneficiaries to deliver contraceptives at the doorstep, that is,

- INR 1 for a pack of 3 condoms

- INR 1 for a cycle of OCPs

- INR 2 for a pack of emergency contraceptive pills (ECP)

2. Promotion of spacing methods (interval IUCD)

- Training of health personnel in IUCD insertion at all levels of health facilities
- Ensure availability of IUCD CuT380 A
- Ensuring IUCD services on fixed days at all sub centres and PHCs
- CHC, SDH and DH will provide regular IUCD insertion services
- Strengthen the counselling system at the facilities with high case load

3. STERILIZATION SERVICES

- Promotion of non-scalpel vasectomy for increasing male participation
- Emphasis on Minilap tubectomy services
- Accreditation of private providers and NGOs for service delivery
- Increasing the pool of trained service providers (Minilap, Laparoscopic sterilization and non-scalpel vasectomy)
- Operationalising fixed day centers for sterilization is an essential step in this direction



4. Comprehensive abortion care

- Manual Vacuum Aspiration (MVA) facilities and medical methods of abortion in 24 X 7 Primary Health Centres.
- The comprehensive Medical Termination of Pregnancy (MTP) services are to be made available at all District Hospitals and Sub-district level hospitals with priority given to „delivery points".
- Capacity building of Medical Officers, to equip them with skills necessary to provide safe abortion services at PHC level and above.
- Medical abortion drugs (Mifepristone + Misoprostol for upto 7 weeks and Ethacridine lactate for 12 to 20 weeks)are to be included in the essential drug list

5. Management of sexually transmitted and reproductive tract infections (RTI and STI)

- RTI/STI services to be provided at all CHCs and FRUs and at 24 X 7 PHCs.
- For Syndromic Management availability of colour-coded kits, RPR testing kits for syphilis and HIV test should be ensured first at delivery points
- Service providers should be trained in Syndromic Management of STI and RTI.
- Importantly services should be made available across entire reproductive age group including adolescents, youth and adults.

Health Systems Strengthening for RMNCH+A Services

- The **key steps** proposed for strengthening health facilities for delivery of RMNCH+A interventions are as follows:

a) Prepare and implement facility specific plans for ensuring quality and meeting service guarantees as specified under IPHS

b) Assess the need for new infrastructure, extension of existing infrastructure on the basis of patient load and location of facility



Health Systems Strengthening for RMNCH+A Services

c) Equip health facilities to support forty-eight-hour stay of mother and newborn.

d) Engage private facilities for family planning services, management of sick newborns and children, and pregnancy complications.

e) Strengthen referral mechanisms between facilities at various levels and communities.

f) Provision for adequate infrastructure for waste management

Resources

- The creation of regular posts under state government so that contractual appointments can be slowly reduced and sustainable HR structure is developed

- **Strengthening sub centres through additional human resources:** In sub centres of remote and hilly area, will have 2 ANMs, 1 male multipurpose worker, 1 pharmacist and 1AYUSH doctor

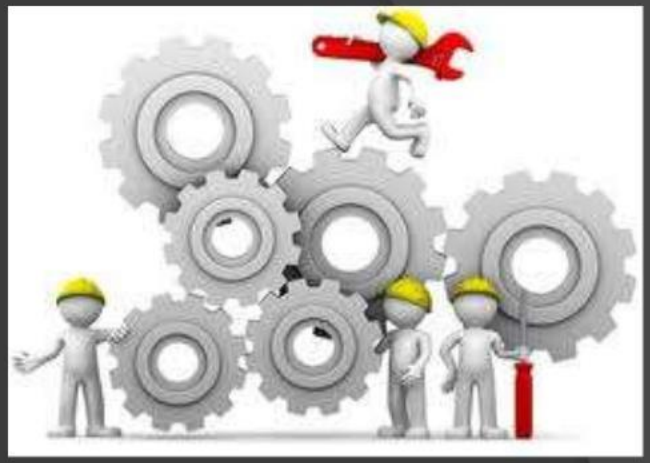
- Capacity building of MO for reproductive, adolescent, maternal, newborn and child health

- Training of nurses and ANM for SBA, IMNCI, Navjaat Shishu Suraksha Karyakram and IUCD insertion

Policies on drugs, procurement system and logistics management

- Availability of free generic drugs for out/in patients in public health facilities is to be made by states for minimising out of pocket expenses.
- Rational prescriptions and use of drugs
- Timely procurement of drugs and consumables
- Distribution of drugs to facilities from DH to sub centre; and uninterrupted availability to patients is to be ensured.
- Placing essential drug lists (EDL) in the public domain
- Computerised drugs and logistics MIS system

Quality assurance



- Quality assurance at all levels of service delivery

- Quality certification/ accreditation of facilities and services
  - Certification for achievement of Indian Public Health Standards
  - Certification should be on comprehensive quality assurance for both infrastructure and service delivery
  - Recommended that health facilities should be first certified by District and State Quality Assurance Cells

Community participation

- Engage **Village Health Sanitation and Nutrition Committees** and **Rogi Kalyan Samiti**

- **Utilize the Village Health and Nutrition Days** as a platform for assured and predictable package of outreach services

- **Social audit**: social audits can be centred around activities like
    - i) Conduct of maternal death audits via verbal autopsies
    - ii) Utilization of health facility checklists

Monitoring, Information & Evaluation Systems

- **Civil registration system:** Efforts to ensure 100% registration of births and deaths under Civil Registration System.



- **Web enabled Mother and Child Tracking System (MCTS):** A more recent initiative is to link MCTS with AADHAR in order to track subsidies to eligible women

Monitoring, Information & Evaluation Systems

- **Maternal Death Review (MDR):** identify causes of maternal deaths and the gaps in service delivery
- **Perinatal and Child Death Review:** Death reports with cause of death for any child under five
- **Review missions:** Annual Joint Review Missions by the RCH Division and Common Review Missions under NRHM

Monitoring, Information & Evaluation Systems

- **Health Management Information System (HMIS) based monitoring and review:**

Following things monitored and interpreted at National, State & District levels:

- ✓ Full Antenatal Care
- ✓ Institutional Delivery,
- ✓ Sterilization procedure
- ✓ IUCD insertion
- ✓ Full Immunization
- ✓ Child & Maternal Death.

Ensuring results and accountability

- **MCTS** — Web-based Mother and Child Tracking System
- **HMIS** — Web-based Health Management Information System for monitoring service delivery indicators
- **AHS** — Annual Health Survey conducted in 9 high burden states each year, both performance & outcome indicators measured
- **SRS** — Sample Registration System : Data brought out by Registrar General of India on key outcome indicators at the national & state level

Delivery Points

- L1 - minimum of 3
- L2 - minimum of 10
- L3 - minimum of 20 – 50

- MCH Wing

Score Card

Score Card:
HMIS Based Dashboard Monitoring System

Score card: HMIS based score card captures only service delivery indicators and assists in comparative assessment of state and district performance

- **16 indicators** selected based on life cycle approach (RMNCH+A) representing various phases
- State average is the reference point for each indicator ; Each indicator is scored based on its contribution towards the state average: Positive scores (> state average)|Negative scores (< state average)
- Indicators score aggregated as district score (all indicators given same weightage)
- Districts classified into **four categories** based on total score ;
  Total score for a district can range between +64 to -64 (4 X 16 indicators)

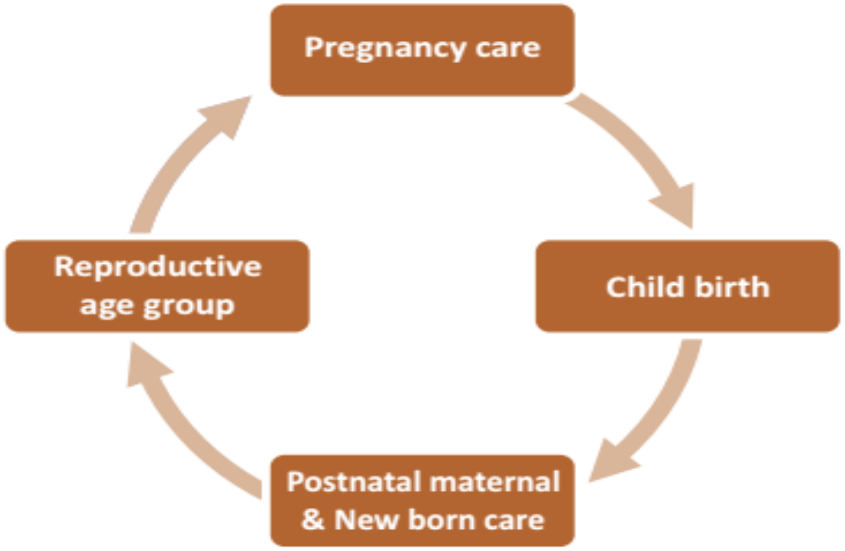
Choice of Indicators for Dash board across the life cycle

Proportion of:

- 1st Trimester registration to total ANC registration
- Pregnant women received 3 ANC to total ANC registration
- Pregnant women given 100 IFA to total ANC registration
- Cases of pregnant women with Obstetric Complications and attended to reported deliveries
- Pregnant women receiving TT2 or Booster to total ANC registration

Proportion of:

- Post-partum sterilization to total female sterilization
- Male sterilization to total sterilization
- IUD insertions in public plus private accredited institutions to all family planning methods (IUD plus permanent)



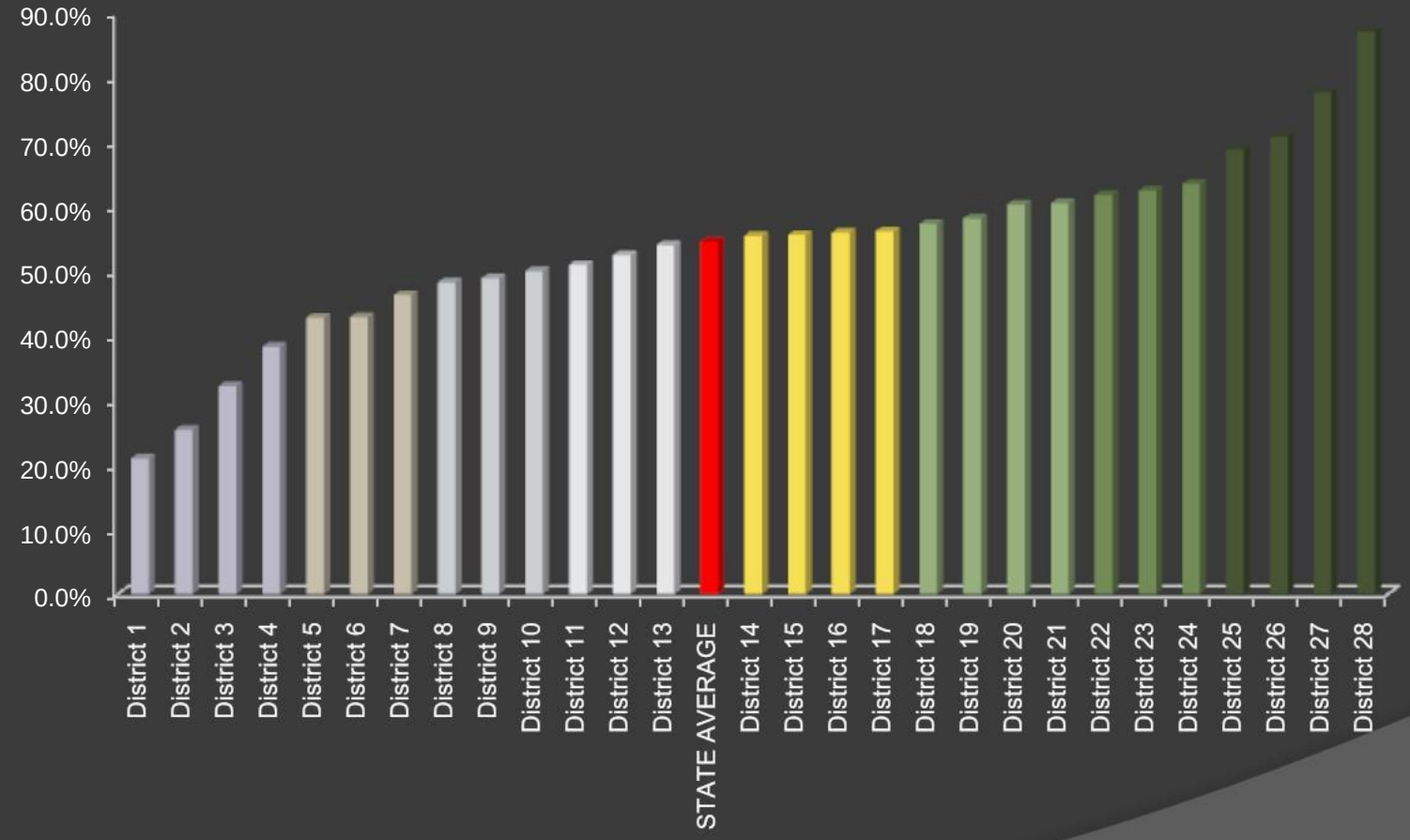
Proportion of:

- SBA attended home deliveries to total reported home deliveries
- Institutional deliveries to total ANC registration
- C-Section to reported deliveries

Proportion of:

- Newborns breast fed within 1 hour to total live births
- Women discharged in less than 48 hours of delivery in public institutions to total number of deliveries in public institutions
- Newborns weighing less than 2.5 kg to newborns weighed at birth
- Newborns visited within 24hrs of home delivery to total reported home deliveries
- Infants 0 to 11 months old who received Measles vaccine to reported live births

EXAMPLE – Scoring for an indicator



Survey-based RMNCH score card for India

National Score Card (16 RMNCH indicators)

		Indicators		Non-NE High Focus States										NE States								Non High Focus States									
				Bihar	Chhattisgarh	Himachal Pradesh	Jammu & Kashmir	Jharkhand	Madhya Pradesh	Orissa	Rajasthan	Uttar Pradesh	Uttarakhand	Arunachal Pradesh	Assam	Manipur	Meghalaya	Mizoram	Nagaland	Tripura	Sikkim	Andhra Pradesh	Goa	Gujarat	Haryana	Karnataka	Kerala	Maharashtra	Punjab	Tamil Nadu	West Bengal
Outcomes	Mortality	Under-five mortality rate -SRS 2010	59	64	41	40	46	59	92	78	69	79			83							46		56	55	45	15	33	45	27	37
		Infant mortality rate - SRS 2012	44	44	48	38	41	39	59	57	52	57	36	32	55	11	12	24	15	28	28	43	11	41	44	21	12	23	30	22	32
		Neonatal mortality rate - SRS 2010	33	31	27	31	31	28	44	42	40	42			55							30		31	33	25	7	23	29	16	29
		Maternal mortality ratio - SRS (2007-09)	212	261	269			201	269	258	318	359	359		360							134		148	153	178	81	104	172	87	145
	Fertility	Total Fertility Rate - SRS 2010	2.5	3.7	2.8	1.8	2.0	3.0	3.2	2.5	2.1	3.5			2.5							1.8		2.5	2.2	2.0	1.8	1.9	1.8	1.7	1.8
		Births to women aged 15-19 out of the total births-DLHS 3 (2007-08)	5.0	9.2	7.3	0.9	1.2	5.9	5.3	4.8	4.7	8.3	1.7	2.5	5.2	1.0	3.0	2.9		5.4	6.0	10.4	2.9	5.4	4.3	10.7	2.9	9.7	5.7	5.2	10.9
	Nutrition	Newborns with birth weight less than 2.5 kg - NHS 2010		22.4	15.8			36.3	28.9	22.3	38.7	28.2	24.1		26.1																
		Children under 5 years who are underweight- NFHS 3 (2005-06)	45.9	56.4	52.1	36.2	26.4	59.2	60.3	44.3	44.2	47.3	56.0	36.9	40.4	23.8	46.3	21.8	26.7	31.6	39.2	36.5	26.3	47.4	41.9	41.1	28.8	39.7	27.0	35.2	40.3
	Gender	Child sex ratio 0-6 years - Census 2011	914	933	964	909	859	943	912	934	883	888	886	960	957	954	970	971	964	966	953	943	920	885	890	943	859	883	946	946	950
Adequate Coverage	Cross-cutting	Full Immunization Coverage (1) - CES 2009	61.0	49.0	57.9	75.8	46.9	38.7	42.8	59.5	55.8	40.9	71.9	24.8	59.1	51.9	60.9	73.7	27.8	85.1	66.0	68.0	87.8	56.9	71.7	79.2	83.3	78.5	83.8	77.5	64.9
		Household having access to toilet facility- DLHS 3 (2007-08)	40.7	17.0	17.9	55.0	65.2	14.5	22.3	16.9	25.1	20.9	19.2	66.7	65.9	66.3	66.2	66.2		91.9	93.3	78.4	77.3	93.5	56.1	67.2	98.7	47.8	75.0	86.3	56.3
		Couple using spacing method for more than 6 months- DLHS3 (2007-08)	9.2	2.8	2.9	12.2	13.2	4.3	5.0	9.2	6.0	8.8	13.6	15.3	17.0	11.8	5.3	15.6		28.4	20.2	0.8	10.2	8.6	13.4	9.2	4.9	8.5	23.7	9.0	16.5
	Diarrhoea	ORT or Increased fluids for diarrhea (2) CES 2009	53.0	40.4	66.3	57.4	79.8	56.4	45.9	69.0	46.9	28.2	57.0	64.6	65.0	82.9	66.6	60.4	57.5	65.5	76.4	54.8	81.8	63.0	79.8	74.0	76.2	58.1	53.3	68.3	48.0
	Pneumonia	Care seeking for ARI in any health facility (3) CES 2009	62.0	62.9	65.0	62.8	65.7	70.9	75.9	76.4	80.9	72.3	65.2	64.9	70.2	77.8	84.3	67.8	50.0	91.2	85.5	88.2	97.0	84.4	62.2	60.7	84.2	93.1	94.7	88.8	77.8
	Service Delivery	Women who received 4+ ANC - CES 2009	53.1	14.7	41.0	44.6	76.9	25.7	39.0	58.9	27.8	25.4	59.6	29.9	59.5	61.1	42.9	65.7	11.8	85.8	80.8	62.6	85.5	73.1	57.2	78.2	90.1	85.1	48.3	89.0	80.8
		Deliveries attended by S&A (4) CES 2009	76.2	53.2	56.4	93.7	82.9	47.9	82.9	76.1	79.8	64.2	58.7	71.9	65.5	82.7	85.2	85.2	41.8	88.9	83.1	95.0	96.8	85.2	68.3	68.8	66.6	85.5	66.7	96.0	72.6
		Mothers who received care within 2 days of delivery for their last birth (5)- NFHS 3 (2005-06)	35.4	15.9	25.5	40.8	48.0	17.0	27.9	16.3	28.9	14.2	50.2	25.5	13.8	48.1	28.8	51.5	11.9	68.5	26.7	69.8	78.4	54.0	40.4	63.0	87.7	58.8	55.5	89.9	57.8
		Early initiation of Breast Feeding (<1hr) - CES 2009	33.5	16.3	44.8	58.8	20.4	15.6	31.3	33.7	27.7	23.6	24.8	35.6	46.1	75.3	67.7	56.5	40.4	55.7	7.1	37.2	66.3	50.0	53.0	58.2	66.0	51.3	16.4	59.1	28.2
		Exclusive breast feeding for 6 months (among 6-9 months children) - CES 2009	56.8	51.8	47.4	53.0	79.4	36.3	46.2	46.8	55.2	36.3	38.7	48.4	50.7	53.2	45.1	52.9	35.8	65.6	68.3	52.2	91.8	44.7	54.3	49.8	45.3	57.2	49.3	20.9	28.7

Indicators Remark:

1. Children (12-23 months) receiving: 1 dose BCG, 3 doses of DPT/DPTc each and 1 measles vaccine
2. Among children <2 years of age who had diarrhea in preceding 2 weeks
3. Among children <2 years of age who had ARI in preceding 2 weeks
4. Delivery by Doctor, ANM/Nurse/LHV
5. Postnatal care from a doctor/nurse/LHV/ANM/other health personnel

Colour Codes:

Mortality Indicators, Nutrition, Fertility: Green - Less than 20% of the national average,
Yellow - 20% below and above National Average
Red - More than 20% of the National Average

Other Indicators: Green - More than 20% of the national average
Yellow - 20% below and above National Average
Red - Less than 20% of the National Average

White - Data not available

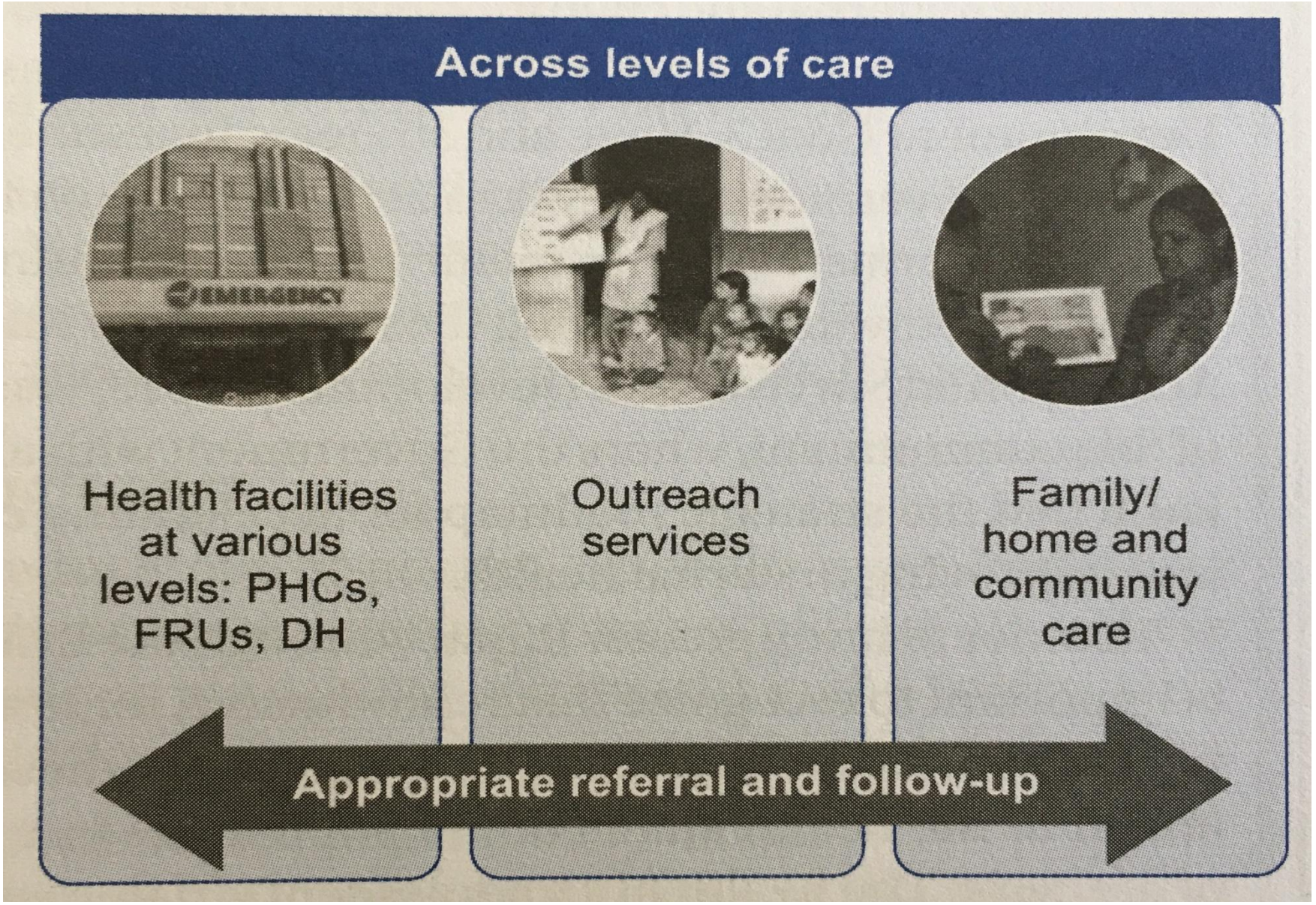
Causes of under-five deaths, 2010



Child mortality rate



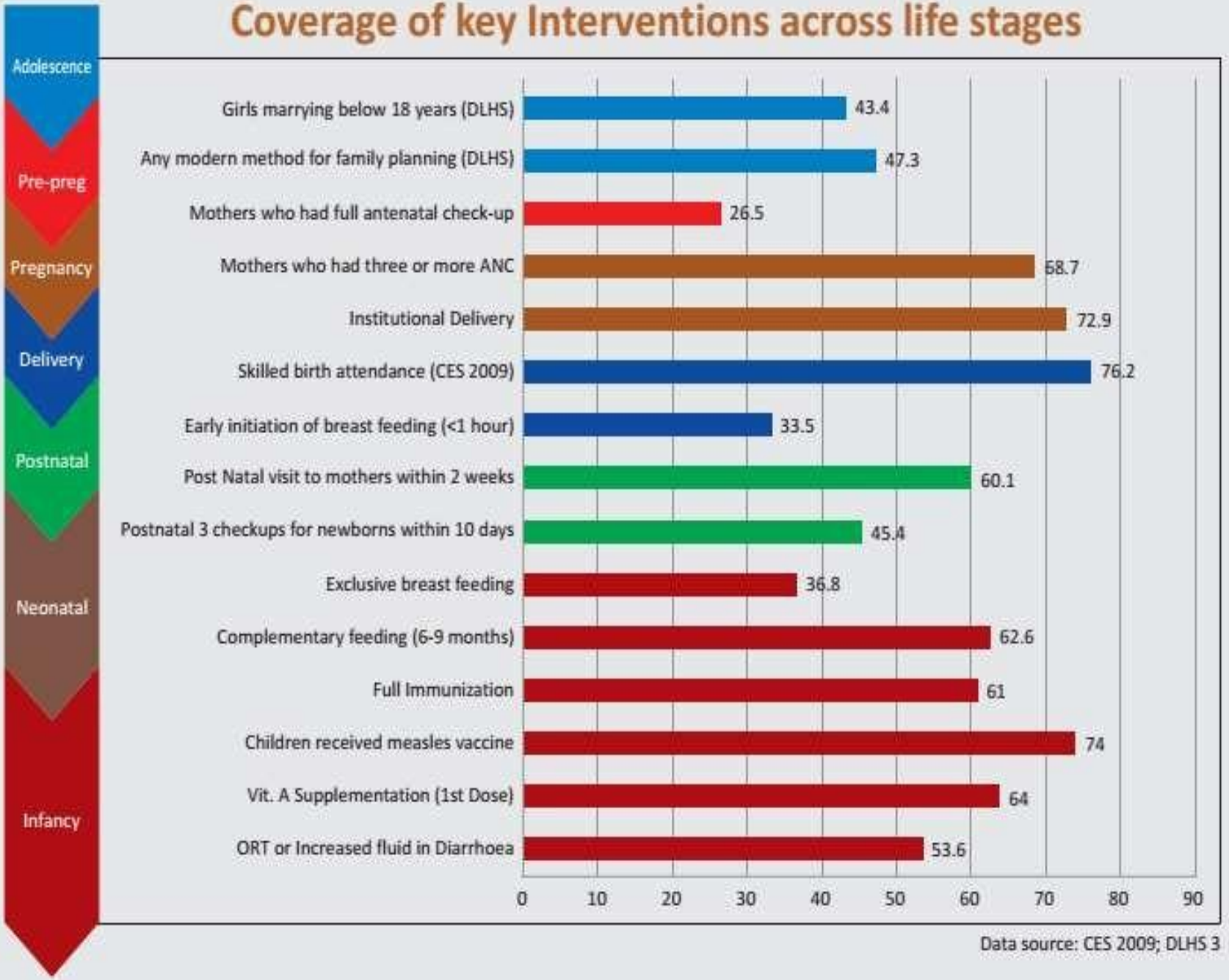
Interventions across Levels of Care



Interventions across Life Stages

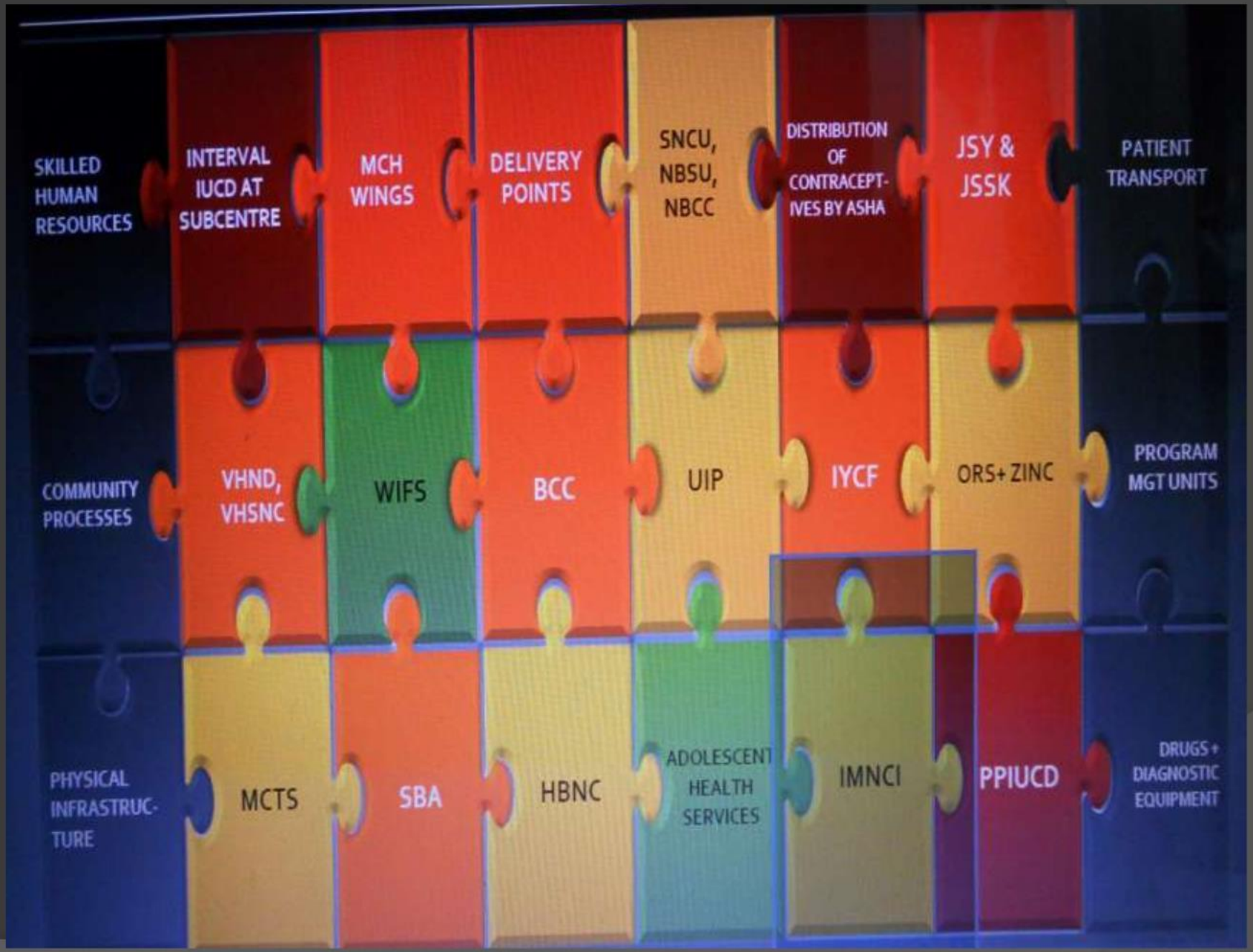


Coverage of key Interventions across life stages



Data source: CES 2009; DLHS 3

Source : DLHS 3 and CES 2009


SKILLED HUMAN RESOURCES
INTERVAL IUCD AT SUBCENTRE
MCH WINGS
DELIVERY POINTS
SNCU, NBSU, NBCC
DISTRIBUTION OF CONTRACEPT-IVES BY ASHA
JSY & JSSK
PATIENT TRANSPORT
COMMUNITY PROCESSES
VHND, VHSNC
WIFS
BCC
UIP
IYCF
ORS+ ZINC
PROGRAM MGT UNITS
PHYSICAL INFRASTRUC-TURE
MCTS
SBA
HBNC
ADOLESCENT HEALTH SERVICES
IMNCI
PPIUCD
DRUGS + DIAGNOSTIC EQUIPMENT

Student Assignment

(Answer all for 10 marks main question)

1. **What is MMR & IMR ? How RMNCH + A strategies can help India to improve these indices ?**

2. **Explain in detail “Anemia Mukt Bharat” program of Govt. of India.**

3. **What is “Breast Crawl” & KMC ? Explain its use in Indian scenario.**

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Thank You